INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Paclitaxel · Days 1, 8 and 15 of 28

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00621
Version 3

Paclitaxel 80 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Day 22 is a planned week off, not a fourth routine dose. Continue according to benefit and tolerance. The NCCP indications are off-label in Ireland and require the local consent / approval process.

Treatment stages

01 **Paclitaxel**
28-day interval

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathing symptoms, severe diarrhoea, inability to drink, painful swollen limbs, unusual bleeding, or worsening numbness. Tell staff immediately about infusion-site pain or an allergic reaction.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How the medicines work

The aim and duration depend on the primary cancer and exact NCCP indication. Confirm the cancer-specific plan before treatment; this shared medicine schedule does not replace that discussion.

TAXANE CHEMOTHERAPY

Paclitaxel

Taxol (conventional paclitaxel)



Locks the cell division machinery

Cells use tiny structures called microtubules to separate their chromosomes when dividing. Paclitaxel stabilises these structures so they cannot rearrange normally, interrupting cell division. It can also affect nerves and other healthy tissues.

Microtubules form → Structures remain locked → Division is interrupted

What to watch for

Report tingling or numbness before the next dose, especially if it affects walking or hand use. Tell the nurse immediately about flushing, chest or throat tightness, itching, dizziness or breathing changes during infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim

Indication

Source-defined advanced gastric, breast, ovarian, small-cell lung or bladder indications

Schedule and phase changes

Paclitaxel 80 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Day 22 is a planned week off, not a fourth routine dose. Continue according to benefit and tolerance. The NCCP indications are off-label in Ireland and require the local consent / approval process.

Evidence in context

The source provides regimen-specific references. No numerical efficacy estimate is transferred from a different dose, tumour type or schedule.

Why this treatment may be chosen

Paclitaxel may slow or shrink metastatic breast cancer and help symptoms. This prescribed pattern includes a week without paclitaxel. There is no verified numerical survival estimate for this exact schedule in this review; results from a different weekly pattern do not predict your outcome.

The treating team considers the cancer, previous treatment, health and your priorities. Trial averages do not predict an individual outcome.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

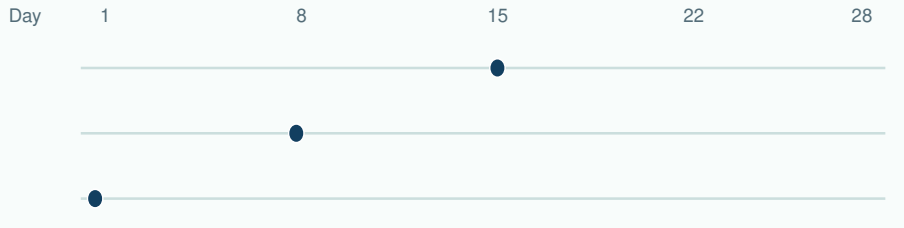
Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Paclitaxel · Days 1, 8 and 15 of 28
NCCP 00621

The phases and how they connect

Paclitaxel

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

28-DAY INTERVAL

Paclitaxel

Repeated cycles

Paclitaxel 80 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Day 22 is a planned week off, not a fourth routine dose. Continue according to benefit and tolerance. The NCCP indications are off-label in Ireland and require the local consent / approval process.

Each phase has its own medicines, checks and supportive prescription. A change of phase does not restart the cardiac monitoring clock.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Milestones show order, not equal time intervals or booked appointments. The treating team requests tests, reviews results and confirms treatment. full blood count (FBC), renal and liver profile before each Day 1, Day 8 and Day 15 treatment, not only at the start of the 28-day block. Assess neuropathy and function before doses. Day 22 is a planned week without paclitaxel in 00621. The separate 00226 weekly alternative must not silently add a fourth dose to this card.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00621 v3 · official regimen · HPRA conventional paclitaxel product information; 09 January 2026

THE WHOLE TREATMENT JOURNEY

Paclitaxel · Days 1, 8 and 15 of 28: treatment and checks

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Treatment plan NCCP 00621	Confirm indication, goal and current prescription	Three weekly doses and a planned week without treatment	Continue according to response, tolerance and prescribed course	Team review before restart or dose change	Agree completion or next treatment
Blood / kidney / liver NCCP 00621 · SMPC	Baseline FBC, kidney and liver tests	FBC, kidney and liver tests before Days 1, 8 and 15	FBC, kidney and liver tests before Days 1, 8 and 15	Recheck recovery and dose before restarting	Follow unresolved abnormalities
Symptoms / medicines NCCP 00621 · SMPC	Review symptoms, medicines and previous treatment	Review effects before each dose	Continue symptom and medicine reviews	Report problems; do not change or restart doses yourself	Agree recovery and next treatment plan
Heart / electrocardiogram (ECG) / echo NCCP 00621 · SMPC	Cardiac history, symptoms and infusion observations guide assessment. Do not add the trastuzumab three-month echo schedule to paclitaxel monotherapy.	Review symptoms and indicated tests	Continue the risk- and treatment-specific plan	Urgent assessment for chest pain, fainting or breathlessness	Agree follow-up from exposure and results
Response / course NCCP 00621 · SMPC	Confirm cancer-specific treatment aim	Three weekly doses and a planned week without treatment	Review benefit, symptoms and the agreed scan plan	Reassess if treatment is held or symptoms change	Agree continuation, change or completion
Bone health / other care NCCP 00621 · SMPC	Review previous/ ongoing treatments and fracture risk	No automatic chemotherapy-cycle dual-energy X-ray absorptiometry (bone density) (DXA)	Keep any separate endocrine/ fracture-risk plan	Review if clinical risks change	Continue separately indicated follow-up

What each check is for

1



Treatment and safety review

Before each planned treatment or four-week review for olaparib

FBC, renal and liver profile before each Day 1, Day 8 and Day 15 treatment, not only at the start of the 28-day block. Assess neuropathy and function before doses.

Ask / act: The treating team confirms results and the plan; this map does not authorise a dose.

2

Benefit and continuation

At agreed clinical review points

Paclitaxel may slow or shrink metastatic breast cancer and help symptoms. This prescribed pattern includes a week without paclitaxel. There is no verified numerical survival estimate for this exact schedule in this review; results from a different weekly pattern do not predict your outcome.

Ask / act: Discuss benefit, toxicity and priorities before continuing or changing treatment.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan



STEP 1

Before the first visit

Blood tests, relevant cardiac assessment, medicines review, fertility discussion and the consent conversation come first.



STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Staff check the line, give prescribed premedicines and treatment, and observe for reactions. Tell them immediately about burning, pain, rash or breathlessness.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

Phase-specific visit pattern

Infusion and observation times differ by drug and first versus later doses; the unit confirms the full visit duration, including blood review and preparation.

Checks through the course

A named team member should arrange and review each test.

Blood tests

FBC, renal and liver profile before each Day 1, 8 and 15 dose; not only at the start of the 28-day block.

Heart tests

No routine fixed-cycle echo or ECG is mandated for this regimen alone; assess symptoms and individual cardiac risk. At a change or completion, agree cardiac follow-up from treatment exposure, symptoms and results.

Cancer imaging

Agree response imaging and any tumour markers for the actual primary cancer, disease sites and treatment aim. The metastatic breast 2–4-month example is not a universal interval for lung, prostate, gastric, ovarian or bladder cancer.

The wider cancer-specific plan

This medicine schedule serves several cancers; confirm which NCCP indication applies.

Local treatments

Surgery, radiotherapy and any endocrine / androgen-deprivation treatment follow the actual primary cancer and treatment aim. No breast surgery or HER2 phase is automatically added.

Bone health

Bone-health / dual-energy X-ray absorptiometry (bone density) (DEXA) requirements depend on endocrine or androgen-deprivation treatment, fracture risk and the primary cancer plan; not on this taxane alone.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Paclitaxel

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Paclitaxel

Treatment or check	Timing in this view
Chemotherapy · Day 15	 **Day 15** intravenous (IV)
Chemotherapy · Day 8	 **Day 8** IV
Chemotherapy · Day 1	 **Day 1** IV
Planned week off	 **Days 22–28** No routine Day 22 paclitaxel
Administration reaction	 **Days 1–28** Report during or after any visit
Nausea / appetite	 **Days 1–5** May occur early or later
Blood counts / infection	 **Days 1–28** Risk varies; fever needs prompt advice
Fatigue / mouth changes	 **Days 1–28** Variable timing and severity

Treatment or check	Timing in this view
Nerves / skin / nails	1 — 28 **Days 1–28** Can build across doses

No dates to enter: The chart explains the pattern. Your team books actual appointments and reviews results.

Treatment delays: A delayed dose does not automatically delay an echo or other test that is already due.

Phase changes: Do not change the schedule using this educational map.

> **Why these bands stay broad:** These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00621 v3 · official regimen ↗

> **There are no guaranteed "safe days".** Fever, shivers, sudden illness, severe breathing changes or chest pain need prompt assessment on any day.

Between-treatment notes

Day 1–28: Your current reference cycle

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Support around the medicines

1 The correct phase prescription

Taxanes use low-risk antiemetic support; hypersensitivity premedication is a separate requirement.

2 Premedicines and growth factors

Paclitaxel premedicines differ between weekly and fortnightly dosing. Use the phase-specific prescription.

3 Tests and appointments

The team should name the booking owner, due window and person reviewing each required test. Tell them if an expected appointment has not arrived.

4 After chemotherapy

Discuss recovery, fertility, rehabilitation and the next treatment or surveillance step.

5

Review the full medicine plan

First-dose NCCP suggested premedication: dexamethasone 8 mg IV, chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before paclitaxel. From Day 8, dexamethasone may be omitted in the source suggested table; add or modify for increased risk or previous reaction under consultant guidance. Famotidine omission is also a consultant/local-policy decision requiring close observation. The conventional paclitaxel product information has a different generic premedication regimen. Reconcile the exact product, weekly NCCP pathway and local policy; never self-omit premedication. Low emetic risk; keep the antiemetic plan separate from reaction premedication. Consider analgesia for myalgia/arthralgia and early action on neuropathy.

Benefit, course and continuing checks

Heart checks and the full course

Cardiac history, symptoms and infusion observations guide assessment. Do not add the trastuzumab three-month echo schedule to paclitaxel monotherapy. Day 22 is a planned week without paclitaxel in 00621. The separate 00226 weekly alternative must not silently add a fourth dose to this card.

Questions worth asking

When are heart checks due?

No routine fixed-cycle echo or ECG is mandated for this regimen alone; assess symptoms and individual cardiac risk. Test timing follows the treatment, clinical findings and local pathway.

Where do surgery and radiotherapy fit?

Surgery and radiation follow your actual cancer-specific treatment plan. This shared taxane card does not impose an adjuvant breast sequence.

Does a port mean a pump at home?

No. This regimen has no routine take-home infusion pump. A port is an access device; the team removes the needle after the visit when appropriate and arranges device removal separately.

Try explaining the plan back: Which phase are you in, what changes next, and who is arranging and reviewing your next blood test, echo or scan?

Paclitaxel

- Low blood counts and infection
- Numbness / tingling, sometimes lasting
- Hair loss, muscle or joint pain
- Infusion reaction
- Mouth, bowel, skin and nail effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Paclitaxel · Days 1, 8 and 15 of 28 · reviewed supportive care

Active medicines: Paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).
- From Day 8 onward: chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before treatment. Routine dexamethasone may be omitted after the first dose; prior reaction or increased risk requires consultant review.
- Famotidine may be omitted under the consultant/local plan if appropriate. A prior reaction requires a separate rechallenge assessment; do not reuse the standard draft automatically.
- First-dose NCCP suggested premedication: dexamethasone 8 mg IV, chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before paclitaxel.
- From Day 8, dexamethasone may be omitted in the source suggested table; add or modify for increased risk or previous reaction under consultant guidance. Famotidine omission is also a consultant/local-policy decision requiring close observation.
- The conventional paclitaxel product information has a different generic premedication regimen. Reconcile the exact product, weekly NCCP pathway and local policy; never self-omit premedication.
- Low emetic risk; keep the antiemetic plan separate from reaction premedication. Consider analgesia for myalgia/arthralgia and early action on neuropathy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Paclitaxel · Days 1, 8 and 15 of 28

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Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hands / feet

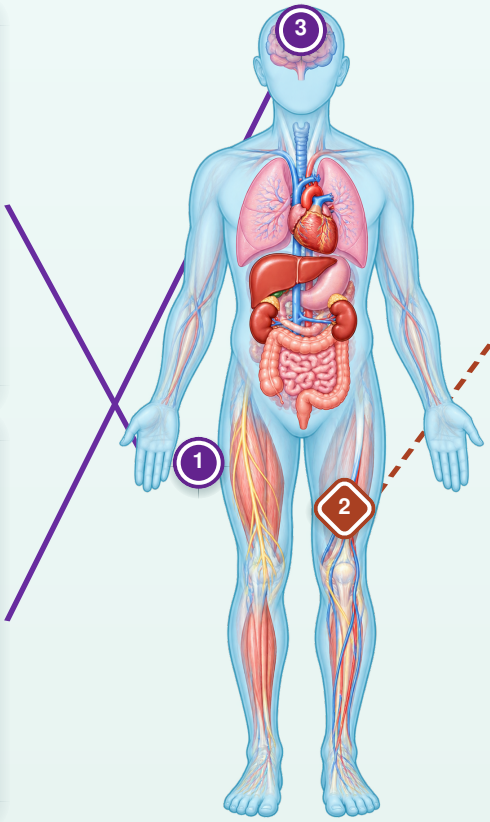
Paclitaxel

Report numbness or tingling before the next dose.

3 Hair / scalp

Chemotherapy phase

Hair loss is common. Ask about scalp cooling before treatment where suitable.



2 Blood / infection

Chemotherapy phase

Fever, shivers or suddenly unwell: call immediately.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Selected medicine-specific effects; the full risk discussion includes hair loss, fertility and other individual risks.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00621 v3 · official regimen**
   https://healthservice.hse.ie/documents/6854/621_V3_Paclitaxel_80_zanF78k.pdf
2. **HPRA conventional paclitaxel product information; 09 January 2026**
   https://assets.hpra.ie/products/Human/27560/Licence_PA2315-115-001_09012026104101.pdf
3. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
4. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
5. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
6. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
7. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
8. **NICE NG 101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
9. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
10. **NCCP NCIS antiemetics v 8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
11. **ASCO · biomarkers in metastatic breast cancer**
    https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
12. **ESMO metastatic breast cancer guideline · 2026**
    https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
13. **NCCP antiemetic guidance; reconcile with regimen and individual prescription**
    https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf